OFFICE OF THE UNIVERSITY REGISTRAR & ACADEMIC SENATE

ACADEMIC QUALITY ASSURANCE, EXAMINATIONS & STUDENT WELFARE GOVERNANCE FRAMEWORK

ATTENDANCE AND MEDICAL ABSENCE ADMINISTRATION POLICY

Document Code:	**POL-ACAD-AT-02**	**Version & Status:**	Version 3.2 (2025 Mandatory Senate Revision)
Effective Date:	01 January 2025	**Supersedes:**	Section 5.2 of Legacy Student Handbook (HBK-ACAD-PR-05 v1.8, 2023)
Approving Body:	University Senate & Academic Affairs Committee	**Linked Policies:**	POL-ACAD-EX-01, POL-ACAD-CW-03, POL-MED-WF-04, HBK-ACAD-PR-05

SECTION 1: STATUTORY ATTENDANCE REQUIREMENT AND RECORDING MECHANISMS

Clause 1.1 (Mandatory 80% Baseline Attendance Threshold): Regular participation in scheduled academic activities is a foundational requirement for degree accreditation. Every full-time undergraduate and postgraduate student must maintain a minimum cumulative attendance of **eighty percent (80%)** in each registered module across all scheduled lectures, tutorials, workshops, and practical sessions during a fifteen-week teaching semester to be eligible to sit the Final Examination.

Clause 1.2 (Supersession of 2023 Attendance Floor): Effective 01 January 2025, this policy (POL-ACAD-AT-02 v3.2) supersedes Clause 5.2 of the 2023 Student Handbook (`HBK-ACAD-PR-05` v1.8). Whereas the 2023 Handbook permitted medical attendance condonation down to 60%, the 2025 Senate Revision establishes a standard medical condonation zone of **70%–79%** and an absolute hard floor of **65%** (strictly restricted to inpatient hospitalization under Clause 3.1). Any student falling below 65% attendance is automatically barred from final examinations (`NQ / Barred`).

ATTENDANCE GOVERNANCE DEFINITIONS (PART A)

- **Standard Eligibility (80%–100%):** Automatic clearance to download the Final Examination Admission Card without administrative intervention.
- **Medical Condonation Zone (70%–79%):** Attendance deficit excusable by the Head of Department upon submission of Form ATT-02A and CMO-verified medical certificates.
- **Hospitalization Waiver Zone (65%–69%):** Exceptional condonation requiring Dean's approval, Inpatient Hospital Discharge Summary (>= 7 days), and Form ATT-02B.

ATTENDANCE GOVERNANCE DEFINITIONS (PART B)

- **Mandatory De-Registration (< 65%):** Absolute regulatory floor. Students below 65% cannot sit final exams regardless of reason and must apply for Semester Intercalation.
- **Form ATT-02A:** Standard Medical Absence & Attendance Condonation Request Form (submitted within 5 working days of return to classes).
- **Form ATT-02B:** Extended Hospitalization Academic Catch-Up & Dean's Attendance Waiver Agreement.

SECTION 2: MEDICAL ABSENCE REPORTING AND CONDONATION bands (70% TO 79%)

Clause 2.1 (Important Principle — Medical Certificates Do Not Convert 'Absent' Marks to 'Present'): Students must understand that submitting a medical certificate does not artificially inflate raw biometric attendance percentages in the Learning Management System (LMS). Instead, verified medical certificates authorize the Registry to reclassify unexcused absences as *Medically Justified Absences (MJA)*, enabling the student to qualify for formal **Attendance Condonation** when their raw attendance falls below the 80% statutory threshold.

Clause 2.2 (Procedure for Standard Medical Condonation — 70% to 79% Raw Attendance): Where a student's raw module attendance at Week 14 falls between **70.0% and 79.9%** due to documented illness, the deficit shall be condoned by the Head of Department provided the student has complied with the following requirements:

Sub-clause 2.2.1 (a): For every medical absence exceeding **two (2) consecutive teaching days**, the student must obtain an SLMC-registered medical certificate and have it verified by the University Chief Medical Officer (`Form MED-04-CLR` under `POL-MED-WF-04`).

Sub-clause 2.2.1 (b): Submit the completed *Medical Absence Condonation Form (Form ATT-02A)* together with the CMO Clearance Slip to the Faculty Academic Administration Office within **five (5) working days of returning to academic classes** (and no later than the Friday of Teaching Week 14).

Table 2.1: University Attendance Bands, Final Examination Eligibility, and Condonation Authority (2025 Rules)

Raw Attendance Band	Examination Eligibility Status	Approving Authority	Mandatory Forms & Medical Evidence	Academic Consequence if Unapproved
80.0% – 100.0%	Automatically Eligible	Automated LMS Clearance	None required (self-certification up to 2 single days)	Full access to Examination Admission Card
70.0% – 79.9%	Eligible via Standard Medical Condonation	Head of Department (HoD)	Form ATT-02A + Form MED-04-CLR (SLMC Medical Certificate submitted within 5 working days of return)	Barred from Final Exam (Grade: NQ) if medicals not filed on time
65.0% – 69.9%	Eligible ONLY via Exceptional Hospitalization Waiver	Dean of Faculty	Form ATT-02B + Inpatient Hospital Discharge Summary (>= 7 days) + Form MED-04-CLR + Completed Catch-Up Plan	Barred from Final Exam (Grade: NQ); must repeat module
Below 65.0% (< 65%)	STRICTLY INELIGIBLE (Hard Regulatory Floor)	No Waiver Permitted (Senate Rule)	Form PRG-05-INT (Medical Intercalation / Semester Deferral under HBK-ACAD-PR-05)	Automatic De-registration from Final Exam; transferred to Intercalation

SECTION 3: EXTENDED HOSPITALIZATION WAIVER (65%–69%) AND THE 65% HARD FLOOR

Clause 3.1 (Exceptional Dean's Waiver for Inpatient Hospitalization — 65.0% to 69.9%): A student who experiences an extended medical emergency requiring continuous inpatient hospitalization of **seven (7) or more calendar days** (for example, a 10-day hospitalization during the teaching semester) may see their raw attendance drop below 70%. Where raw attendance remains between **65.0% and 69.9%**, the Dean of the Faculty may grant an *Exceptional Hospitalization Attendance Waiver* subject to three mandatory conditions:

Sub-clause 3.1.1 (a): Submission of `Form ATT-02B` within **five (5) working days of hospital discharge**, attached to the official Hospital Admission & Discharge Summary and CMO Verification Slip (`Form MED-04-CLR`).

Sub-clause 3.1.1 (b): Completion of a structured *Academic Catch-Up & Guided Study Plan* designed by the Module Leader, including review of recorded lecture captures and submission of formative tutorial exercises prior to Week 15.

Sub-clause 3.1.1 (c): Full clearance of all mandatory laboratory/practical safety competencies under Section 4.

Clause 3.2 (Absolute Prohibition on Waivers Below 65.0%): Under no circumstances may a Head of Department or Dean waive attendance where a student's attendance falls below **65.0%** (equivalent to missing more than 5 weeks of a 15-week semester). Where severe, prolonged medical illness causes attendance to drop below 65%, the student is protected from academic failure through *Temporary Medical Intercalation (Semester Deferral without Tuition Penalty)* under Policy `HBK-ACAD-PR-05` Section 4.

Table 3.1: Impact of Hospitalization Duration on 15-Week Semester Attendance & Required Administrative Protocol

Hospitalization Duration	Typical Attendance Drop (15-Wk Semester)	Resulting Attendance (Assuming 95% Prior)	Applicable Regulatory Track	Mandatory Student Action Within 5 Working Days of Discharge
1 to 3 Calendar Days	-2% to -4%	91% – 93% (Above 80%)	Routine Medical Logging	Log absence on LMS; retain medical note in case of future illness
4 to 6 Calendar Days	-6% to -9%	86% – 89% (or 75%–79% if prior absences exist)	Standard Condonation Track (Clause 2.2)	Obtain Form MED-04-CLR at UMC + file Form ATT-02A with Faculty Office
7 to 14 Calendar Days (e.g., 10-Day Hospitalization)	-13% to -18%	77% (HoD Track) or 66%–69% (Dean Waiver Track)	Standard (70–79%) or Exceptional Dean's Waiver (65–69%)	File Form ATT-02A/B + Hospital Discharge Summary + Form MED-04-CLR + Catch-Up Plan
15 to 21 Calendar Days	-20% to -28%	65% – 72%	Dean's Waiver (if >=65%) or Intercalation Review	Mandatory meeting with Academic Advisor + Faculty Welfare Officer
Exceeding 21 Teaching Days	Greater than -30%	Below 65% (< 65% Hard Floor)	Mandatory Medical Intercalation (HBK-ACAD-PR-05)	File Form PRG-05-INT for unpenalized Semester Deferral

SECTION 4: MANDATORY LABORATORY, CLINICAL, AND STUDIO PRACTICAL ATTENDANCE

Clause 4.1 (Separate Tracking of Practical Components): For modules incorporating wet laboratories, engineering workshops, computing studios, or clinical placements, practical attendance is tracked independently from lecture attendance. Students must complete a minimum of **85% of scheduled laboratory sessions** (and 100% of designated health-and-safety competency practicals) regardless of overall lecture attendance.

Clause 4.2 (Make-Up Laboratory Sessions for Hospitalized Students): A student who misses a mandatory laboratory session due to documented hospitalization or acute illness must submit a *Laboratory Make-Up Request (Form LAB-MK-01)* to the Module Laboratory Coordinator within **five (5) working days** of returning to campus. Each department schedules designated Consolidation & Make-Up Lab Slots during Teaching Weeks 13 and 14 so hospitalized students can fulfill practical competencies without penalty.

Table 4.1: Component-Specific Attendance Thresholds and Remediation Mechanisms

Instructional Component	Minimum Required Attendance	Medical Condonation Floor	Remediation Mechanism for Hospitalized Students	Form Reference
Theory Lectures	80% Cumulative	70% (HoD) / 65% (Dean)	Review Panopto/LMS Lecture Captures + Guided Reading Summary	Form ATT-02A / ATT-02B
Small-Group Tutorials	80% Cumulative	70% (HoD) / 65% (Dean)	Submit written solutions to missed tutorial problem sets to Tutor	Form ATT-02A
Laboratory / Engineering Practicals	85% Practical Sessions	75% ONLY with Make-Up Lab	Attend Week 13/14 Consolidation Make-Up Lab Session + submit lab logbook	Form LAB-MK-01
Clinical / Field Placements	100% Mandatory Hours	No reduction in total hours	Complete compensatory clinical hours during inter-semester vacation window	Form CLIN-COMP-03

SECTION 5: AUTOMATED ATTENDANCE ALERTS, BARRED LISTS, AND APPEALS

Clause 5.1 (Three-Stage Automated Early Warning System): The University LMS calculates rolling attendance every Friday at 17:00 and dispatches automated notifications to students and Academic Advisors according to Table 5.1.

Clause 5.2 (Publication of the Provisional Attendance Bar List & 5-Day Appeal Window): On the Monday of Teaching Week 15, the Registrar publishes the *Provisional Examination De-Registration (Barred / NQ) List*. Any student listed on the Provisional Bar List who possesses valid, previously unrecorded medical condonation evidence (`Form MED-04-CLR`) must submit an *Emergency Attendance Bar Appeal (Form APP-AT-03)* to the Faculty Registrar within **five (5) working days** of list publication. No attendance appeals are accepted once Final Examinations commence.

Table 5.1: Automated LMS Attendance Warning Escalation and Administrative Deadlines

Alert Level	Trigger Threshold	Automated System Action	Required Student Response	Escalation Deadline
Stage 1: Amber Advisory	Attendance drops below 85%	Automated email & Portal alert to student	Verify biometric logs; submit any pending medical certificates	Within 5 working days
Stage 2: Red Warning	Attendance drops below 80%	Alert sent to Student, Module Leader & Academic Advisor	File Form ATT-02A + Form MED-04-CLR with Faculty Office immediately	Before Friday of Week 14
Stage 3: Critical Alert	Attendance between 65% and 70%	Mandatory Welfare Hold placed on Exam Card download	File Form ATT-02B (Dean's Waiver) + Hospital Discharge Summary	Before Friday of Week 14
Stage 4: Provisional NQ Bar	Uncondoned <80% or Raw <65% at Week 15	Listed on Provisional Examination Bar List (Grade: NQ)	File Appeal Form APP-AT-03 (only if >=65% with valid medicals)	Within 5 working days of Bar List

POLICY CONFLICT & SUPERSESSION NOTICE (2025 vs. 2023 HANDBOOK)
Conflict Alert: Legacy Clause 5.2 of the 2023 Student Handbook (HBK-ACAD-PR-05 v1.8) states that medical attendance condonation may be granted down to 60% attendance. That threshold is SUPERSEDED by POL-ACAD-AT-02 v3.2 (2025). Under current 2025 regulations, standard medical condonation applies to 70%–79%, exceptional hospitalization waivers apply to 65%–69%, and 65.0% is the absolute non-negotiable minimum attendance floor.